ONE Clinical Nutrition & Hydration Protocol

POSSIBLE F-TAG/QMS

Quality of Care:

- ADL dependency
- Nutrition
- Hydration
- Pressure Ulcers
- Urinary Continence

Dietary:

- Nourishing Diet
- Meeting Nutritional needs
- Assistive Devices
- Therapeutic Diets

Quality Measures

- Falls with Major Injury
- UTI
- Pressure Ulcers
- Help with ADL
- Walking/Locomotion
- Antipsychotic Use
- ER & Hospital Visits

THERAPIES

- SLP
- OT

PURPOSE OF THERAPY

- To decrease risk for dehydration which prevents a multitude of medical conditions including nutritional, skin and urinary issues.
- To improve safe and efficient intake of fluids in order to decrease risk for adverse events
- To decrease risk for unintended or unwanted weight loss or other nutritional deficiencies which can prevent a multitude of medical conditions including nutritional, skin and urinary issues
- To improve safe and efficient intake to decrease risk for adverse events

GOAL EXAMPLES

SLP:

- Patient will safely swallow thin liquids via cup as evidenced by timely swallow and no change in vocal quality on 8/10 opportunities in order to safely advance diet to thin liquids
- Patient will follow free water protocol with written cues to improve hydration and safety.
- Patient will participate in oral care with caregivers prior to taking thin liquids in order to decrease risk of adverse events in all thin liquid intake opportunities.
- Patient will consume ____cc of thin water using safe swallow strategies with___ cues on 8/10 opportunities in order to improve safety with liquid intake.
- Patient will ask for water from caregivers in 3/5 opportunities with min verbal cues in order to decrease risk for dehydration.
- Patient will use written reminder to drink sips of water throughout day to total at least ___cc of fluids in order to decrease risk for dehydration.
- Patient will improve endurance for meal as evidenced by RPE of < 3 on Borg scale during meal in order to improve endurance, safety and nutrition during PO intake.

TREATMENT IDEAS

- **Free Water Protocol**
  - Determine who is on thickened liquids and/or NPO
  - Obtain orders for assessment and therapy
  - Conduct instrumental studies as needed
  - Provide therapy to improve swallow and advance to thin liquids OR Set up Free Water Protocol
  - DC therapy OR continue on a maintenance program as indicated
  - Training of staff to administer
  - Use of maintenance therapy to assure safety and adequate intake
  - Use of conjunction with water protocol to improve hydration
  - Use of different colors to incorporate following directions, attention to task, recall, etc...
- **Restorative swallow therapy including**
  - Resistive exercise
  - RMST
  - Viscosity and volume changes to bolus for exercise
  - NMES/SEmg
- **Compensatory Swallow Therapy including**
  - Analysis of possible strategies
  - Implementation of strategies
  - Caregiver Education
  - Maintenance program as needed

POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
Quality Measures Antipsychotic Medications/Behavior Symptoms Affecting others: medications are used most often to decrease behavioral symptoms; can lead to: increased cost for medications, can lead to increased confusion, can lead to need for increased nursing supervision (QM category: Received Antipsychotics Medication) Antianxiety/Hypnotic/Depression Symptoms: medications are used most often to decrease depressive/anxiety symptoms; can lead to: increased cost for medications, can lead to increased confusion, can lead to need for increased nursing supervision due to drowsiness/decreased participation in therapy, can lead to increased social isolation (QM category: Received Antipsychotics Medication) Falls with Major Injury			Patient will demonstrate clear oral cavity post swallow on 8/10 regular trials in order to improve swallow function and safely upgrade to regular foods. Patient will follow safe swallow strategies with written cue card and occasional reminders from staff to improve swallow safety and efficiency in all intake opportunities. Patient will attend to meal with minimal cues from caregivers in order to achieve nutritional goals **OT:** Patient will lift cup/utensil to mouth without assistance from caregivers to improve self-feeding. Patient will improve toileting skills to Mod I to improve independence with ADLs and decrease avoidance of fluids Patient will drink three 20-ounce mugs of water per day for improved bladder health and to decrease episodes of UI to 2x/day. Patient will increase grip strength to _____ to maintain functional grasp needed for bringing cup to mouth for hydration. Patient will improve B elbow Flexion strength/ROM to _____ to promote functional grasp and UE coordination to bring cup to mouth for dehydration prevention.	**Cognitive-Communication Goals** • Recall of swallow strategies • Prospective memory to request water or other liquids • Tracking of daily hydration /nutrition • Making choices on a menu • ID of food groups **Self-Feeding tasks to improve ability to take cup IND** **Strengthening of UE to improve ability to take cup** **Fine motor tasks to improve self-feeding of Jelly Drops** **Sequencing of simple cooking/meal prep** **Cognitive tasks for adding Hydration into daily schedule** **Analysis of need for adaptive drinking devices to improve independence with fluid intake** **Treatment to address toileting skills and efficiency to decrease avoidance of liquids** **Visual scanning and attention skills to improve access to hydration** **Oral care routine as part of OT treatments to promote hygiene and increase FMC and GMC for grasping grooming and hygiene products in order to preventaspiration pneumonia and promote readiness for free water protocol for hydration.**

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				Promote hand strengthening activities for maintaining grip strength in order to better monitor signs of decline in strength that correlates with posterior tongue strength and risk for dysphagia **ROM activities in relation for self-feeding movements to promote coordination and functional reach with grasping of cup. Standardized Tests to Utilize:** • Arm curl test (relate to ROM and strength needed to bring cup to mouth for hydration) • Box and Blocks Test (focus of functional pinchand movement in a size related to jelly drops) • Hole Peg Test (can be used to promote FMC dexterity needed in manipulation of grooming items for oral care) • Monitor grip strength (measurable for signs of decline in strength can indicated dysphagia and screening for SLP may be indicated) • Barthel Index